Module 1
Learn With PsycLearn Essentials

Module 2
Opening to Depressive and Bipolar Disorders

Lesson 1: Culture and Mood Disorders
- Focus of the Course — Reading • 2 min
- Classifying Psychopathology — Reading • 4 min
- Culture and Psychopathology — Reading • 4 min
- Culture and Intersectionality — Reading • 4 min
- Culture and Stigma — Reading • 4 min
- Check Your Understanding: Culture and Mood Disorders — Practice Assignment • Grade: 100%

Lesson 2: Introduction to Depressive and Bipolar Disorders
- Depressive and Bipolar Disorders — Video • 6 min
- Living with a Mood Disorder: A Case Study — Reading • 10 min
- Overview of Depressive and Bipolar Disorders — Reading • 10 min
- Check Your Understanding: Introduction to Depressive and Bipolar Disorders — Practice Assignment • Grade: 80%

Key Takeaways
- Key Takeaways: Opening to Depressive and Bipolar Disorders — Reading • 10 min

Module 2 Quiz
- Module Quiz: Opening to Depressive and Bipolar Disorders — Graded Assignment • Grade: 87.5%

Module 3
Depressive and Bipolar Disorders

Lesson 3: Overview of Depressive Disorders
- Depression: A Case Study — Reading • 5 min
- What Does Depression Feel Like? — Reading • 7 min
- **Diagnostic Criteria and Prevalence for Depressive Disorders** — Reading • 5 min
- Check Your Understanding: Overview of Depressive Disorders — Practice Assignment • 12 min

Lesson 4: Overview of Bipolar Disorders
- Bipolar: A Case Study — Reading • 5 min
- What Does Bipolar Feel Like? — Reading • 7 min
- Diagnostic Criteria and Prevalence for Bipolar Disorders — Reading • 5 min
- Check Your Understanding: Overview of Bipolar Disorders — Practice Assignment • 13 min

Key Takeaways
- Key Takeaways: Depressive and Bipolar Disorders — Reading • 10 min

Module 3 Quiz
- Module Quiz: Depressive and Bipolar Disorders — Graded Assignment • 16 min

Module 4
Biological Contributing Factors

Module 5
Psychological Contributing Factors

Module 6
Treatments for Depressive and Bipolar Disorders

Module 7
Suicide

Module 8
Conclusion

Module 9
Cumulative Quiz

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Course Resources

Module 11
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Diagnostic Criteria and Prevalence for Depressive Disorders

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Diagnostic Criteria and Prevalence

Two common disorders listed in *ICD-11* involving depression are the spectrum of depressive disorders and dysthymic disorder. In addition, there are several other psychological conditions that include symptoms of depression, such as premenstrual dysphoric disorder and a spectrum of disorders associated with pregnancy, childbirth or the puerperium, popularly known as postpartum depression. In this section, we will highlight several that have social relevance or that you are likely to encounter in the popular media.

Before launching into the description of each, it's important to note that some are listed as primary disorders, such as dysthymic disorder or premenstrual dysphoric disorder, and some as a subtype (also called a specifier) of a primary disorder, such as substance-induced mood disorder or seasonal affective disorder, which are both listed under the spectrum of depressive disorders.

Depressive Disorders

Depressive disorders are mood disorders in which a person experiences one or more depressive episodes, characterized by persistent sadness or loss of interest in activities previously enjoyed, nearly every day for two weeks. Additional symptoms may include

- a loss of interest/pleasure in activities the person used to enjoy;
- notable weight loss or gain;
- trouble sleeping (insomnia) or sleeping too much (hypersomnia);
- feeling agitated/restless or slowed down;
- decreased energy, fatigue, exhaustion;
- feelings of worthlessness or guilt;
- problems concentrating and making decisions; and
- recurrent thoughts of death, suicidal ideation, or attempted suicide.

Within *ICD-11*, depressive disorders are further identified by the number of episodes (singular or recurrent episodes), level of severity (mild, moderate, or severe), and whether they include symptoms of psychosis, which is a loss of contact with external reality, such as delusions and hallucinations. Within *DSM-5*, severe depression with recurrent episodes and symptoms of psychosis would be categorized as major depressive disorder, although *ICD-11* recognizes it within a wider spectrum of depressive disorders.

Substance-induced mood disorders are listed as a specifier under the spectrum of depressive disorders by *ICD-11* and include depressive symptoms that are specifically linked to the use of, exposure to, or withdrawal from a substance. Individuals diagnosed with this class of disorder experience symptoms believed to be caused directly by the physiological effects of a substance, which may be a drug of abuse (e.g., alcohol, cocaine, opioids, etc.), a medicinal drug, or a heavy metal or toxin (e.g., gasoline, paint, an organophosphate insecticide, etc.).

The accuracy of estimates on the prevalence of depression varies by country and region and is influenced by the level of access to health care, a person's age, and their level of income (World Health Organization, 2023b). More than 75% of people in low-income and middle-income countries live without treatment for depression (World Health Organization, 2023a). It is estimated that depressive disorders account for about 3.8% of the global population and 5% of the adult population. Of the adults, prevalence is estimated to be about 4% among adult men and 6% among adult women. Among people over the age of 60, prevalence is estimated to be about 5.7%. Women are 50% more commonly diagnosed with depression than men.

Also listed as a specifier by *ICD-11*, and popularly referred to as **seasonal affective disorder**, is a recurrent mood disorder with depressive symptoms that has a distinct seasonal pattern. The important point is that the symptoms of depression are not attributable to a recurrent social event, such as school schedule or seasonal unemployment, but rather to changes in the seasons.

Research has suggested that less daylight in the fall and winter months contribute to this disorder. Specifically, studies showed that places with a greater difference in the number of daylight hours between the summer and winter (meaning, those closer to Earth's poles) had a higher number of cases than did those with a consistent amount of light year-round (those closer to the equator) (e.g., Rosen et al., 1990). However, some research did not observe a relationship between depression and "latitude, season, or sunlight" (Traffanstedt et al., 2016; Winthorst et al., 2017). These researchers suggest that seasonal affective disorder is attributable to folk psychology (Traffanstedt et al., 2016, p. 825). However, bright light therapy (i.e., daily exposure of patients to light) has been shown to be an effective treatment for seasonal affective disorder, as well as other types of depression (Campbell et al., 2017), implicating a common mechanism underlying these depressive disorders.

Prevalence rates for seasonal affective disorder vary depending on latitude, gender, and age (Freed et al., 2010). Rates in Canada, The Netherlands, and the United Kingdom were estimated to be 4.8%, 3%, and 2.4%, respectively (Axelsson et al, 2002; Mersch et al., 1999; Michalak et al., 2001).

Dysthymic Disorder

Dysthymic disorder, which may also be referred to as persistent depressive disorder in *DSM-5*, is a longer term depressive disorder, with symptoms that last for at least two years, and within that time period, the symptoms persist for at least two months at a time, causing distress and disruption in everyday life. The other symptoms associated with dysthymic disorder are similar to but less severe than those associated with the spectrum of depressive disorders. For example, people diagnosed with dysthymic disorder experience thoughts of death, suicidal ideation, or attempted suicide at a significantly lower prevalence rate than those with recurrent depressive disorders, but at a higher prevelance rate than those from the general public (Witte et al., 2010). Individuals with dysthymic disorder often describe themselves as being "down in the dumps" (Parker & Malhi, 2019). Further, many individuals with dysthymic disorder, when asked how long they have felt depressed, will respond by saying, "I have always felt like this," especially when the onset of their depression was in childhood (Parker & Malhi, 2019).

While some major depressive episodes can last many months or years, most don't. Studies show that the average length of a severe depressive episode is approximately three months. At that point, the episode tends to end, even in the absence of treatment (Eaton et al., 2008; Spijker et al., 2002). For half of people with a depressive disorder, once their first episode ends, they never have another. About 35% of people with a depressive disorder have recurring depression. Their first episode ends in three months on average, but then they have multiple additional episodes over the course of their life. For 15% - 20% of people, the major depression doesn't go away at all (Eaton et al., 2008).

Fewer studies have investigated the prevalence of dysthymic disorder and, therefore, estimates are more variable and less reliable (Parker & Malhi, 2019). However, given these limitations, the prevalence rate of dysthymic disorder for the global population is estimated to be about 1.5% with about 1.3% among adult men and 1.8% among adult women. As with depressive disorders, the prevalence rate of dysthymia among women is higher globally (Vos, 2012). Within the U.S. population, prevalence rates are about 1% for men and 1.9% for women (National Institute of Mental Health, 2020).

Premenstrual Dysphoric Disorder

Premenstrual dysphoric disorder is associated with the menstrual cycle; people with this disorder display psychological symptoms, such as moodiness, irritability or anger, feelings of depression or anxiety, and decreased interest in usual activities, physical symptoms, such as lethargy and pain, or cognitive symptoms, such as difficulty concentrating. Symptoms begin a few days prior to the menstrual period, improve a few days following the beginning of the menstrual period, and dissipate about one week later.

For a diagnosis, the symptoms must be more debilitating than the typical premenstrual symptoms many people experience, and negatively affect the person's quality of life and ability to function in multiple settings (Reid & Soares, 2018). Premenstrual dysphoric disorder can be distinguished from other depressive disorders by absence of symptoms one week following the end of menses. The prevalence rate for premenstrual dysphoric disorder is about 6% for reproductive age women in the U.S. population (Yonkers & Simoni, 2018).

The addition to the *ICD-11* in 2022 has spurred controversy. Inclusion is advocated by patients, psychiatrists, and the drug industry as recognizing an important mood disorder needing research and treatment. However, its inclusion is criticized by psychologists and women's rights activists who feared that it could be overdiagnosed, that women's normal hormonal changes would be pathologized, and that it would be used to silence women by labeling ordinary symptoms of life as a mental disorder. It has also been argued that inclusion of premenstrual dysphoric disorder was pushed by drug companies seeking profits (Schroll & Lauritsen, 2022).

Post(Peri)partum Depression

Mental or behavioral disorders associated with pregnancy, childbirth or the puerperium is popularly referred to as **postpartum depression** but more accurately as **peripartum depression**. These disorders are characterized by syndromes during pregnancy or the puerperium (also called the postpartum), which is the period of time 6 weeks following birth when the gestational mother's reproductive organs return to a nonpregnancy condition. Symptoms include depression and significant mental and behavioral events that may or may not include delusions, hallucinations, mania, and elements of psychosis. Additional symptoms may include anxiety, obsessions, and compulsions, but that are not associated with another medical condition or exposure to or withdrawal from substances.

People with postpartum depression may also have thoughts of death and suicide, in addition, they may also contemplate committing infanticide. Note that not every woman diagnosed with postpartum depression will think about infanticide, it's most often people experiencing psychotic delusions (such as the irrational belief that their baby is possessed by the devil) who actually kill their child (American Psychiatric Association, 2022). People who have had a previous episode of postpartum depression or prior diagnoses of a depressive disorder or bipolar disorder are at a greater risk for psychotic delusions.

Many people experience what are called the "baby blues" during or after pregnancy but not all of them develop a diagnosable depression. Because pregnancy and the period surrounding the birth of a child is a time of large changes in a pregnant person's hormonal state and personal and social life, there are many possible causes of peripartum depression. We know that levels of sex hormones such as estrogen and progesterone influence depression (Slavich & Sacher, 2019). Feeling stress, such as the perception that you are not able to cope with the demands being placed on you, is also a strong predictor of depression (LeMoult, 2020). So, while no single cause of peripartum depression has been identified, there are a large number of factors that may play a role.

About 17% of people experience postpartum depression around the world. Wang and colleagues (2021) conducted a meta-analysis that included 172,342 participants from 80 countries to investigate the prevalence rate by country, region, and income. The overall average prevalence rate was 17.22%, with variability. Higher income countries generally, but not consistently, had lower prevalence rates than lower income countries. For example, among higher income countries, the prevalence rate in the United Kingdom was 21.5% but 10.58% in New Zealand, and among lower income countries, the prevalence rate in Egypt was 22.99% but 3% in Ghana. From these results, we can roughly estimate that one in six women will develop postpartum depression during or following pregnancy.

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